

Passages - Being Mortal

The veneration of elders may be gone, but not because it has been replaced by veneration of youth. It's been replaced by veneration of the independent self.

The job of any doctor, Bludau later told me, is to support quality of life, by which he meant two things: as much freedom from the ravages of disease as possible and the retention of enough function for active engagement in the world. Most doctors treat disease and figure that the rest will take care of itself. And if it doesn't--if a patient is becoming infirm and heading toward a nursing home--well, that isn't really a medical problem, is it? To a geriatrician, though, it is a medical problem. People can't stop the aging of their bodies and minds, but there are ways to make it more manageable and to avert at least some of the worst effects.

We end up with institutions that address any number of societal goals--from freeing up hospital beds to taking burdens off families' hands to coping with poverty among the elderly--but never the goal that matters to the people who reside in them: how to make life worth living when we're weak and frail and can't fend for ourselves anymore.

Royce had no sympathy for the individualist view. "The selfish we had always with us," he wrote. "But the divine right to be selfish was never more ingeniously defended." In fact, he argued, human beings need loyalty. It does not necessarily produce happiness, and can even be painful, but we all require devotion to something more than ourselves for our

lives to be endurable. Without it, we have only our desires to guide us, and they are fleeting, capricious, and insatiable. They provide, ultimately, only torment. "By nature, I am a sort of meeting place of countless streams of ancestral tendency. From moment to moment ... I am a collection of impulses," Royce observed. "We cannot see the inner light. Let us try the outer one."

The Emanuels described a third type of doctor-patient relationship, which they called "interpretive." Here the doctor's role is to help patients determine what they want. Interpretive doctors ask, "What is most important to you? What are your worries?" Then, when they know your answers, they tell you about the red pill and the blue pill and which one would most help you achieve your priorities. Experts have come to call this shared decision making.

A bad ending skewed the pain scores upward just as dramatically. Studies in numerous settings have confirmed the Peak-End rule and our neglect of duration of suffering. Research has also shown that the phenomenon applies just as readily to the way people rate pleasurable experiences.

No one ever really has control. Physics and biology and accident ultimately have their way in our lives. But the point is that we are not helpless either. Courage is the strength to recognize both realities. We have room to act, to shape our stories, though as time goes on it is within narrower and narrower confines. A few conclusions become clear when we understand this: that our most cruel failure in how we treat the sick and the aged is

the failure to recognize that they have priorities beyond merely being safe and living longer; that the chance to shape one's story is essential to sustaining meaning in life; that we have the opportunity to refashion our institutions, our culture, and our conversations in ways that transform the possibilities for the last chapters of everyone's lives.

Our ultimate goal, after all, is not a good death but a good life to the very end.

"You're special," she whispered to her.